

BAS SICK CALL PHARMACOLOGY QUIZ

1163rd TFSB, 1st MDTF, JBLM | Battalion Aid Station
50 Multiple-Choice Questions | 24 Medications

INSTRUCTIONS: Circle the single best answer for each question. Do not reference the BAS formulary during this assessment.

Name: _____ Date: _____ Unit/Section: _____

Benzocaine-Dextromethorphan 5mg-7.5mg Lozenge (Cepacol)

Category: Indication & Dose

Q1. What is Cepacol indicated for, and what is the correct dosing interval, maximum daily lozenge limit, and maximum dispense duration?

- A. Nasal congestion and cough; one lozenge every 4–6 hours; max 8/day; up to 3 days
- B. Sore throat and ear pain; one lozenge every 1–2 hours; max 16/day; up to 3 days
- C. Sore throat and cough; one lozenge every 2–4 hours; max 12/day; up to 5 days
- D. Cough and fever; one lozenge every 6 hours; max 6/day; up to 7 days

Ibuprofen 200 mg

Category: Mechanism of Action

Q2. Which best describes the mechanism of action of Ibuprofen?

- A. Selective COX-2 inhibitor that reduces peripheral prostaglandin synthesis only
- B. Binds glucocorticoid receptors to suppress systemic inflammation
- C. Blocks voltage-gated sodium channels to inhibit pain signal transmission
- D. Non-selective COX-1 and COX-2 inhibitor that reduces prostaglandin synthesis

Category: Indication

Q3. Which condition is NOT an authorized indication for Ibuprofen at BAS sick call?

- A. Menstrual cramps and minor muscular aches
- B. Neuropathic pain secondary to shingles
- C. Temporary reduction of fever
- D. Minor backache and headache

Category: Dose, Frequency & Duration

Q4. What is the authorized dose range, maximum daily dose, and dispense duration for Ibuprofen at BAS sick call?

- A. 200–400 mg every 6–8 hours; max 1,200 mg/day; up to 3 days
- B. 400–600 mg every 4 hours; max 3,200 mg/day; up to 2 weeks
- C. 200–400 mg every 4–6 hours; max 2,400 mg/day; up to one week
- D. 200 mg every 4 hours; max 1,600 mg/day; up to 5 days

Acetaminophen (Tylenol) 325 mg

Category: Indication

Q5. What are the authorized indications for Acetaminophen at BAS sick call?

- A. Treatment of osteoarthritis joint pain and neuropathic pain
- B. Temporary relief of minor aches and pains, and reduction of fever
- C. Relief of allergic rhinitis, urticaria, and nasal congestion
- D. Temporary relief of muscle spasm and menstrual cramps only

Category: Dose, Frequency & Duration

Q6. What is the correct dose, maximum daily tablet count, and dispense duration for Acetaminophen 325 mg at BAS sick call?

- A. 1 tablet every 4–6 hours; max 8 tablets/day; up to 5 days
- B. 2 tablets every 6–8 hours; max 6 tablets/day; up to 3 days
- C. 2 tablets every 4–6 hours; max 10 tablets/day; up to one week

D. 1–2 tablets every 4 hours; max 12 tablets/day; up to one week

Category: Special Considerations

Q7. At what ingestion threshold is Acetaminophen considered toxic and high-risk for liver damage in adults?

- A. Greater than 100 mg/kg or 8 g over 24 hours, or greater than 5 g in a single dose
- B. Greater than 150 mg/kg or 12 g over 24 hours, or greater than 7.5 g in a single dose
- C. Greater than 200 mg/kg or 10 g over 24 hours, or greater than 6 g in a single dose
- D. Greater than 50 mg/kg or 6 g over 24 hours, or greater than 3 g in a single dose

Diclofenac 1% Gel

Category: Indication

Q8. Which joints are specifically listed as eligible for Diclofenac 1% Gel treatment at BAS sick call?

- A. Shoulders, hips, and spine
- B. Temporomandibular joint and sternoclavicular joint
- C. Hands, wrists, elbows, knees, ankles, and feet
- D. Any joint with palpable inflammation regardless of location

Category: Side Effects

Q9. What is the most common category of side effects for Diclofenac 1% Gel?

- A. Systemic cardiovascular effects such as hypertension and peripheral edema
- B. GI effects such as ulceration and bleeding equivalent to oral NSAIDs
- C. Respiratory effects such as bronchospasm and rhinitis
- D. Local skin reactions: dryness, redness, itching, rash, scaling, and peeling

Lidocaine 4% Patch

Category: Indication & Dose

Q10. What is the Lidocaine 4% Patch indicated for, and what is the correct application schedule and dispense duration?

- A. Osteoarthritis pain; apply continuously for 24 hours; change daily; up to one week
- B. Neuropathic pain from shingles or minor musculoskeletal pain; apply up to 12 hours within a 24-hour period; dispense up to one week
- C. Burn pain and surgical wounds; apply two patches for up to 8 hours every 8 hours; up to 5 days
- D. Headache and migraine; apply one patch to the forehead for up to 6 hours; up to 3 days

Menthol 4% Topical Gel

Category: Indication

Q11. What is Menthol 4% Topical Gel indicated for?

- A. Treatment of fungal skin infections such as tinea pedis and tinea cruris
- B. Prevention of infection in minor cuts, scrapes, and abrasions
- C. Temporary relief of minor aches and pains of muscles and joints associated with backache, arthritis, strains, bruises, and sprains
- D. Relief of neuropathic pain and burning associated with peripheral neuropathy

Category: Dose, Frequency & Duration

Q12. What is the correct application frequency and dispense duration for Menthol 4% Topical Gel?

- A. Apply twice daily; dispense up to 2 weeks
- B. Apply once daily at bedtime; dispense up to one week
- C. Apply up to 6 times daily; dispense up to 5 days
- D. Apply a thin layer 3–4 times daily; dispense up to one week

Loratadine (Claritin) 10 mg

Category: Indication

Q13. Which condition is an authorized indication for Loratadine at BAS sick call?

- A. Nasal congestion due to the common cold
- B. Insomnia and motion sickness
- C. Epistaxis unresponsive to direct pressure
- D. Pruritus and wheals secondary to urticaria

Category: Dose, Frequency & Duration

Q14. What is the correct dose and maximum dispense duration for Loratadine?

- A. 10 mg twice daily; dispense up to one week
- B. 10 mg once daily; dispense up to 2 weeks
- C. 10 mg every 8 hours; dispense up to 5 days
- D. 5 mg twice daily; dispense up to 3 days

Category: Side Effects

Q15. What are the documented side effects of Loratadine?

- A. Dry mouth, urinary retention, and tachycardia
- B. Palpitations, insomnia, and hypertension
- C. Headache, drowsiness, and fatigue
- D. Rebound congestion and nasal burning after prolonged use

Diphenhydramine (Benadryl) 25 mg

Category: Indication

Q16. Which set of indications is authorized for Diphenhydramine at BAS sick call?

- A. Allergies, urticaria, nasal congestion, and motion sickness
- B. Allergies, urticaria, insomnia, and motion sickness
- C. Allergies and nasal congestion only
- D. Insomnia and motion sickness only

Category: Dose, Frequency & Duration

Q17. What is the correct dose, maximum daily tablet count, and dispense duration for Diphenhydramine 25 mg?

- A. 1 tablet every 6 hours; max 6 tablets/day; up to 3 days
- B. 2 tablets every 8 hours; max 8 tablets/day; up to one week
- C. 1–2 tablets every 4–6 hours; max 12 tablets/day; up to 5 days
- D. 1–2 tablets every 4–6 hours; max 8 tablets/day; up to 5 days

Category: Special Considerations

Q18. Which presentation is consistent with severe Diphenhydramine toxicity from a single dose greater than 1g?

- A. Miosis, bradycardia, excessive secretions, and bronchospasm

- B. Delirium, seizures, psychosis, hypotension, hallucinations, and QT/QRS prolongation
- C. Hyperthermia, rhabdomyolysis, and metabolic acidosis
- D. Liver failure, jaundice, and elevated transaminases

Oxymetazoline (Afrin) 0.05% Nasal Spray

Category: Mechanism of Action

Q19. What is the mechanism of action of Oxymetazoline nasal spray?

- A. Binds glucocorticoid receptors in nasal mucosa to reduce inflammation
- B. Inverse agonist of H1 receptors preventing histamine-triggered nasal congestion
- C. Stimulates alpha receptors causing vasoconstriction of nasal arterioles, reducing swelling
- D. Physically coats nasal mucosa to protect from allergens and irritants

Category: Indication

Q20. In addition to nasal congestion from colds and allergies, what other condition may Oxymetazoline be used to treat at BAS sick call?

- A. Acute bacterial sinusitis
- B. Epistaxis not responsive to direct pressure
- C. Serous otitis media
- D. Excessive earwax requiring irrigation

Category: Dose, Frequency & Duration

Q21. What is the correct dosing interval, maximum duration, and what complication results from exceeding that duration?

- A. Every 4–6 hours; max 5 days; rhinitis atrophicans
- B. Every 6–8 hours; max 7 days; mucosal ulceration and epistaxis
- C. Every 10–12 hours; max 3 days; rebound congestion (rhinitis medicamentosa)
- D. Every 8 hours; max 10 days; adrenal suppression from prolonged alpha agonism

Saline Mist 0.65% Nasal Spray

Category: Mechanism of Action & Indication

Q22. How does Saline Mist work, and what is it indicated for?

- A. Stimulates alpha receptors causing vasoconstriction; indicated for nasal congestion from colds and allergies
- B. Moisturizes and loosens thick nasal mucus; indicated for dry, irritated nasal passages from colds, allergies, low humidity, or irritants
- C. Blocks H1 receptors in nasal mucosa; indicated for allergic rhinitis and hay fever
- D. Coats the nasal mucosa with an anti-inflammatory barrier; indicated for acute sinusitis

Artificial Tears Ophthalmic Solution

Category: Mechanism of Action & Indication

Q23. How do Artificial Tears work, and what are they indicated for?

- A. Block H1 receptors on conjunctival mast cells; indicated for allergic conjunctivitis
- B. Cause vasoconstriction of conjunctival vessels; indicated for reduction of eye redness
- C. Lubricate the ocular surface; indicated for temporary relief of burning, irritation, and discomfort from dry eye or wind and sun exposure
- D. Inhibit COX-2 locally; indicated for post-operative eye inflammation

Guaifenesin (Mucinex) 600 mg ER Tablet

Category: Indication

Q24. What is the pharmacologic classification and approved indication for Guaifenesin at BAS sick call?

- A. Antitussive; suppresses the cough reflex centrally
- B. Decongestant; causes vasoconstriction of respiratory mucosa
- C. Expectorant; thins bronchial secretions to make coughs more productive
- D. Antihistamine; reduces mucus production via H1 blockade

Category: Dose, Frequency & Duration

Q25. What is the correct dose, frequency, and maximum daily tablet count for Guaifenesin 600 mg ER at BAS sick call?

- A. 1–2 tablets every 8 hours; max 6 tablets/day; up to one week
- B. 1 tablet every 12 hours; max 2 tablets/day; up to 5 days
- C. 1–2 tablets every 12 hours; max 4 tablets/day; up to one week
- D. 2 tablets every 6 hours; max 8 tablets/day; up to 3 days

Pseudoephedrine (Sudafed) 30 mg

Category: Mechanism of Action

Q26. How does Pseudoephedrine differ from Oxymetazoline in its mechanism and CNS effects?

- A. Pseudoephedrine is a beta-agonist; Oxymetazoline is an alpha-agonist; both equally cross the blood-brain barrier
- B. Both are alpha-agonists, but Pseudoephedrine more readily crosses the blood-brain barrier causing greater systemic sympathetic activation
- C. Pseudoephedrine blocks H1 receptors centrally; Oxymetazoline stimulates alpha receptors only peripherally
- D. Pseudoephedrine causes vasodilation to relieve congestion; Oxymetazoline causes vasoconstriction

Category: Indication

Q27. What is the indication for Pseudoephedrine at BAS sick call?

- A. Temporary relief of cough and sore throat
- B. Acute treatment of asthma and bronchospasm
- C. Temporary relief of nasal congestion due to the common cold, hay fever, or upper respiratory allergies
- D. Treatment of insomnia caused by nasal congestion

Category: Dose, Frequency & Duration

Q28. What is the correct dose, maximum daily tablet count, and dispense duration for Pseudoephedrine 30 mg?

- A. 1 tablet every 4–6 hours; max 6 tablets/day; up to 5 days
- B. 2 tablets every 4–6 hours; max 8 tablets (240 mg)/day; up to 3 days
- C. 1 tablet every 6 hours; max 4 tablets/day; up to one week
- D. 2 tablets every 6–8 hours; max 6 tablets/day; up to 3 days

Category: Side Effects

Q29. Which side effect profile is most characteristic of Pseudoephedrine?

- A. Drowsiness, dry mouth, and constipation
- B. Rebound congestion, nasal burning, and stinging after prolonged use
- C. Nausea, heartburn, and stomach upset
- D. Nervousness, restlessness, palpitations, insomnia, and hypertension

Category: Special Considerations

Q48. Why is Pseudoephedrine kept behind the pharmacy counter and subject to purchase limits?

- A. It is a Schedule II controlled substance due to its stimulant CNS effects
- B. It has high abuse potential and can be chemically converted to manufacture methamphetamine
- C. It requires pharmacist oversight due to interactions with the most common OTC medications
- D. It causes severe rebound congestion requiring pharmacist counseling before each purchase

Carbamide Peroxide (Debrox) Drops

Category: Indication & Dose

Q30. What is Debrox indicated for, and what is the correct administration technique and duration?

- A. Acute otitis externa; instill 3 drops twice daily for 5 days; no positioning required
- B. Ear pain from otitis media; instill 10 drops once daily while lying flat; up to 3 days
- C. Cerumen removal; tilt head so affected ear faces ceiling, instill 5–10 drops, wait several minutes, tilt head down to drain; twice daily up to 7 days
- D. Cerumen removal; instill 15 drops, plug with cotton overnight, rinse in morning; up to 5 days

Ondansetron (Zofran) 4 mg ODT

Category: Indication & Dose

Q31. What is Ondansetron's regulatory status and indication at BAS sick call, and what is the correct dose, repeat interval, and maximum daily dose?

- A. OTC; nausea and vomiting; one 4 mg tablet every 4–6 hours; max 16 mg/day; up to 5 days
- B. Not OTC but authorized for BAS sick call; nausea and vomiting; one 4 mg tablet dissolved on the tongue, may repeat in 8 hours; max 24 mg/day; up to 3 days
- C. Controlled substance; post-operative nausea; one 4 mg tablet every 12 hours; max 8 mg/day; up to 3 days
- D. Not OTC but authorized for BAS sick call; nausea and vomiting; two 4 mg tablets at onset, may repeat in 4 hours; max 24 mg/day; up to one week

Category: Special Considerations

Q47. Which cardiac risk requires extra caution before administering Ondansetron, and when is an ECG indicated prior to use?

- A. QRS prolongation; ECG required before every dose regardless of risk factors
- B. PR interval prolongation; ECG required only for IV doses above 32 mg
- C. QT interval prolongation with risk of torsades de pointes; ECG indicated if patient has known long QT syndrome, cardiac conditions, electrolyte abnormalities (e.g., hypokalemia), or is on concurrent QT-prolonging medications
- D. Sinus bradycardia; ECG required in patients over age 65 only

Bismuth Subsalicylate (Pepto-Bismol) 262 mg Chewable Tablet

Category: Mechanism of Action

Q32. Which statement best describes the complete mechanism of action of Bismuth Subsalicylate?

- A. Binds H₂ receptors to reduce gastric acid; coats mucosal cells; inhibits gastric motility
- B. Osmotic agent drawing water into the bowel; neutralizes gastric acid; stimulates mucus production
- C. Salicylate inhibits COX reducing fluid into the bowel; coats the GI tract; has bactericidal properties against H. pylori
- D. Blocks 5-HT₃ receptors in the gut; inhibits gastric secretion; coats inflamed mucosa

Category: Indication

Q33. What are the authorized indications for Pepto-Bismol at BAS sick call?

- A. Constipation, bloating, and hemorrhoids
- B. Acid reflux, esophagitis, and peptic ulcer disease
- C. Nausea and vomiting secondary to motion sickness only
- D. Diarrhea, heartburn, indigestion, nausea, and upset stomach

Category: Dose, Frequency & Duration

Q34. What is the correct dosing interval, maximum daily doses, and dispense duration for Pepto-Bismol?

- A. 2 tablets every 4–6 hours; max 4 doses/day; up to 3 days
- B. 2 tablets every 30–60 minutes as needed; max 8 doses (16 tablets)/day; up to 5 days
- C. 1 tablet every 30 minutes; max 12 tablets/day; up to 5 days
- D. 2 tablets every 2 hours; max 6 doses/day; up to one week

Category: Side Effects

Q35. What characteristically benign but alarming side effect should patients be counseled on when taking Pepto-Bismol?

- A. Orange discoloration of the urine due to bismuth excretion
- B. Temporary yellowing of the skin from salicylate accumulation
- C. Darkening of the tongue and/or stool
- D. Metallic taste in the mouth due to bismuth salts

Category: Special Considerations

Q36. A patient is taking doxycycline and wants to use Pepto-Bismol for diarrhea. What is the correct guidance?

- A. Avoid Pepto-Bismol entirely as bismuth forms a toxic complex with tetracyclines
- B. Take both simultaneously since the anti-diarrheal effect outweighs the interaction
- C. Dose doxycycline at least 1 hour before or 3 hours after Pepto-Bismol to maintain antibiotic efficacy
- D. Switch the patient to MiraLax since it has no interaction with tetracyclines

Polyethylene Glycol (MiraLax) 17 g Powder

Category: Indication & Dose

Q37. What is MiraLax indicated for, and what is the correct reconstitution method and dosing schedule?

- A. Diarrhea; dissolve 34g in 16 oz water; once daily; up to 5 days
- B. Constipation; dissolve 17g in 8 oz of any beverage; twice daily until bowel movement then once daily; up to one week
- C. Constipation; dissolve 17g in 4 oz water only; three times daily; up to 3 days
- D. Bloating and gas; dissolve 8.5g in 4 oz water; once daily; up to one week

Famotidine (Pepcid) 20 mg

Category: Indication & Dose

Q38. What is Famotidine indicated for, when should it be taken relative to meals, and what is the maximum daily dose and dispense duration?

- A. Acid reflux and ulcers; take 2 hours after eating; max 80 mg/day; up to 2 weeks
- B. Acid reflux, ulcers, and nausea secondary to anaphylaxis; take 15–60 minutes before eating; max 40 mg/day; up to one week
- C. Acid reflux and gastroparesis; take with meals for best absorption; max 60 mg/day; up to 5 days
- D. Acid reflux only; take at bedtime only; max 20 mg/day; up to 2 weeks

Category: Special Considerations

Q49. What long-term nutritional concern is associated with chronic H2 blocker use?

- A. Hyperphosphatemia due to increased gastric phosphate absorption
- B. Iron overload due to increased intestinal iron absorption in an alkaline environment
- C. Vitamin D deficiency due to impaired fat-soluble vitamin absorption
- D. Vitamin B12 deficiency due to decreased gastric acid impairing absorption

Hydrocortisone 1% Cream

Category: Indication & Dose

Q39. What is Hydrocortisone 1% Cream indicated for, and what is the correct application frequency and dispense duration?

- A. Fungal and bacterial skin infections; apply 4–5 times daily; up to 3 days
- B. Warts, corns, and calluses; apply twice daily; up to 2 weeks
- C. Minor skin irritations, rashes, eczema, insect bites, and poison ivy; apply 3–4 times daily; up to one week
- D. Neuropathic skin pain and dermatitis; apply once daily at bedtime; up to 2 weeks

Clotrimazole 1% Cream

Category: Indication

Q40. Which fungal infections are authorized indications for Clotrimazole 1% Cream at BAS sick call?

- A. Onychomycosis (nail fungus) and tinea capitis (scalp ringworm)
- B. Tinea versicolor and cutaneous candidiasis only
- C. Tinea pedis (athlete's foot), tinea cruris (jock itch), and tinea corporis (ringworm)
- D. Systemic fungal infections presenting with skin manifestations

Category: Dose, Frequency & Duration

Q41. What is the treatment duration difference between athlete's foot and jock itch with Clotrimazole 1% Cream?

- A. Both require 4 weeks of twice-daily application
- B. Athlete's foot requires 2 weeks; jock itch requires 4 weeks
- C. Athlete's foot requires 4 weeks; jock itch requires 2 weeks (both twice daily; continue 1 week after resolution)
- D. Athlete's foot requires 6 weeks; jock itch requires 3 weeks

Calamine-Zinc Oxide 8%-8% Lotion

Category: Indication & Dose

Q42. What is Calamine-Zinc Oxide Lotion indicated for, and what is the correct application technique?

- A. Fungal and bacterial co-infections; rub into skin until absorbed; 3–4 times daily
- B. Pain and itching from poison ivy, insect bites, and minor skin irritations; shake well, apply thin layer with cotton ball to clean dry skin, let dry; as needed up to one week
- C. Infected lacerations and burns; apply thick layer under occlusive bandage; twice daily for 5 days
- D. Warts and corns; apply thin layer after soaking in warm water; twice daily up to 2 weeks

Bacitracin 500 units/gram Ointment

Category: Indication & Dose

Q43. What is Bacitracin indicated for, and what is the correct application frequency and dispense duration?

- A. Treatment of active skin infections and impetigo; apply 4–5 times daily; up to 3 days

- B. Relief of pain and itching from minor skin irritations; apply twice daily; up to one week
- C. Prevention of infection in minor cuts, scrapes, and burns; apply a small amount 1–3 times daily; up to 5 days
- D. Treatment of tinea pedis and ringworm; apply once daily; up to 4 weeks

Category: Special Considerations

Q50. Why is Bacitracin ineffective against Gram-negative bacteria?

- A. Gram-negative bacteria produce beta-lactamase enzymes that break down Bacitracin
- B. Gram-negative bacteria lack peptidoglycan in their cell walls so Bacitracin has no target
- C. Gram-negative bacteria have an outer membrane that protects the cell wall and prevents the drug from entering
- D. Gram-negative bacteria efflux Bacitracin through multi-drug resistance pumps

Salicylic Acid 40% Patches (Compound W)

Category: Indication & Dose

Q44. What are Salicylic Acid 40% Patches indicated for, and what is the maximum treatment duration for warts versus corns/calluses?

- A. Skin tags and seborrheic keratoses; warts up to 4 weeks; corns up to 2 weeks
- B. Common and plantar warts and corns; warts up to 12 weeks; corns/calluses maximum 14 days (change every 48 hours for both)
- C. Warts and calluses; warts up to 6 weeks; corns up to 4 weeks
- D. Plantar warts only; maximum 8 weeks regardless of lesion type

Fluticasone (Flonase) 50 mcg/spray Nasal Spray

Category: Indication

Q45. What is the approved indication for Fluticasone nasal spray at BAS sick call?

- A. Nasal congestion due to the common cold
- B. Acute bacterial sinusitis as an adjunct to antibiotics
- C. Seasonal and perennial allergic rhinitis
- D. Epistaxis and nasal polyp reduction

Category: Dose, Frequency & Duration

Q46. What is the correct dose and dispense duration for Fluticasone nasal spray?

- A. 1 spray each nostril twice daily; dispense up to one week
- B. 3 sprays each nostril once daily; dispense up to 2 weeks
- C. 2 sprays each nostril twice daily; dispense up to 5 days
- D. 2 sprays each nostril once daily; dispense up to 2 weeks